ASSESSMENT MODULE

What to remember

Online Module Quick Reference

Each lesson of the online module is distilled to its four or five essential ideas. This document does not replace the manual, the video capsules or in-class practice. It gives you a reference point before the in-person session and a quick refresher after the course.

The PAS is the common thread running through the entire course. It will be reused in every chapter that follows and in every in-class scenario. Print the PAS form and keep a copy in your kit.

MODULE CONTENTS: SEVEN LESSONS

ASSESSMENT

00 Introduction to the chapter

Six lessons to assess a patient, decide, and organize what comes next

Why this chapter: in the backcountry, no one will take over in the next few minutes. Your assessment must be complete, orderly and written down, because you are the one who will decide what happens next.

1. **Two phases: assess, then act and evacuate.** The first three lessons build clinical judgment (PAS, triage, shock). The next three organize the response (moving, evacuation, communication).
2. **The PAS is your reference from start to finish.** Same sequence for an illness and for a trauma, for one patient and for ten. It keeps you from skipping a step under pressure.
3. **Serial vital signs replace the lab.** Without imaging or blood work, it is the trend in vital signs over time that tells you whether the patient is stabilizing or deteriorating. That is how shock is recognized.
4. **Deciding also means knowing when to wait.** Evacuating at night, in a storm, with an exhausted group can create a second patient. Lesson 05 gives you the criteria for making the call.
5. **Reasoning online, skills in class.** Physical exam, log roll, BEAM, hypothermia wrap and full scenarios are practised in person, PAS form in hand.

LEARNER'S MANUAL	FIELD MANUAL	IN CLASS
Sections 5 and 6, Shock and Patient Assessment · Online encyclopedia	**PAS pyramid and PAS form, pages 23 and 44**	**PAS form handed out, pyramid poster, first scenario**

01 Patient Assessment System (PAS)

The PAS, your reference from the start to the end of the course

> **A fixed sequence, always in the same order.** The pyramid reminds you that the base must be solid before you build upward.

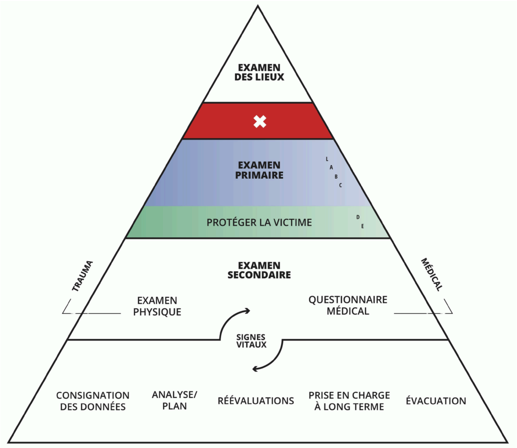

Figure 1 — La pyramide SEP : chaque étage suppose le précédent complété.

1. **Scene survey, before touching the patient.** My safety and the group's, mechanism of injury or nature of illness, number of patients and bystanders, universal precautions, resources and call for help. What you observe on scene (height of fall, position) is objective data: write it down.
2. **Primary survey: what kills in minutes.** Massive hemorrhage controlled first, then airway, breathing, circulation, neurological status (AVPU) and exposure. You treat as you go: a life threat found is a life threat corrected before moving on.
3. **Protect the patient before the secondary survey.** Warmth, insulation from the ground, shelter, position. In the backcountry, the environment becomes a medical problem if you forget it.
4. **Secondary survey: three sets of information.** Vital signs (level of consciousness, respiration, pulse, skin, blood pressure, pupils, temperature), head-to-toe physical exam, SAMPLE history. Injured: physical exam first. Ill: history first. Signs are observed, symptoms are reported.
5. **Record on the PAS form and reassess.** Subjective, objective, assessment (problem list), plan (one plan per problem). Vital signs repeated at regular intervals, by the same person: it is the trend that matters.

LEARNER'S MANUAL
Sections 6.1 to 6.5 and 6.8 · Online encyclopedia

FIELD MANUAL
Pages 23 to 45, from the PAS to documentation

IN CLASS
Scene survey, primary survey, physical exam, SAMPLE, PAS form, hypothermia wrap

02 Triage

Multiple patients, limited resources

> **Triage sorts patients before treating them.** You do not treat the first patient you come across, but the one whose survival depends most on your next few minutes.

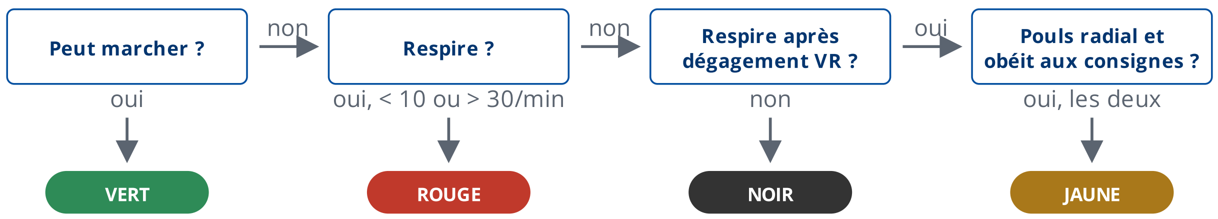

Figure 2 — Algorithme START simplifié (manuel terrain, page 59). Toute autre réponse à la dernière question (pas de pouls radial, retour capillaire de plus de 2 s) : rouge. Les couleurs attribuées peuvent changer : surveillez constamment l'ABC et réévaluez.

1. **Four colours, four priorities.** Red: immediate care, vital functions compromised. Yellow: delayed care, serious condition but can wait a short while. Green: minor injuries, often able to walk. Black: deceased or unsalvageable with the means available.
2. **The START method, three criteria.** Respiration, circulation, level of consciousness. Quick and simple, it brings you almost immediately to the most critical patients.
3. **First step: clear the site.** Ask everyone who can walk to go to a designated point. They are provisionally green and will be reassessed later. Only the patients who cannot move remain on scene.
4. **A difficult but necessary decision.** With multiple patients, a patient who is not breathing after the airway has been opened is tagged black. Time spent on CPR is time taken away from patients who can be saved.
5. **Tag, reassess, delegate.** Improvise tags with whatever you have. Patients' conditions change quickly: reassess all colours, including the greens. The walking wounded can monitor someone's breathing or help carry.

LEARNER'S MANUAL	FIELD MANUAL	LEVEL
Section 6.7, Triage · Online encyclopedia	**Page 59, multiple-casualty triage**	**Awareness level at AWFA 40h: understand the concept, its purpose and its rationale**

03 Shock

Recognizing the progression of a serious injury or illness

> **Shock is inadequate tissue perfusion: the supply of oxygenated blood to the cells is no longer sufficient.** Unrecognized in time, it leads to death. In the backcountry, its most frequent causes are hemorrhage, dehydration and altitude.

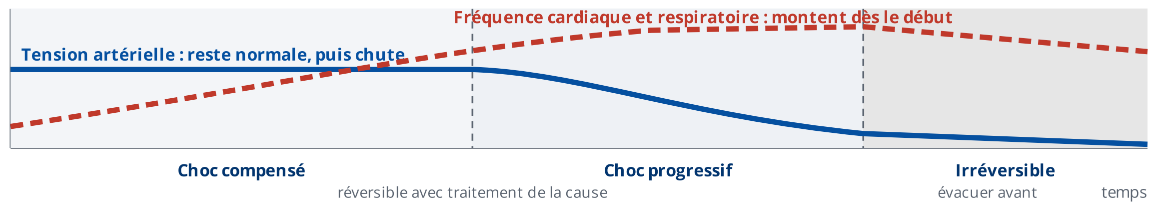

Figure 3 — Une tension normale ne rassure pas : c'est l'accélération du pouls et de la respiration qui signe le choc compensé.

1. **Four main families.** Hypovolemic (hemorrhage, dehydration, burns), cardiogenic (the heart no longer pumps enough), distributive or vasodilatory (anaphylactic, neurogenic, septic, psychogenic) and obstructive or respiratory (oxygen no longer reaches the blood).
2. **Compensated shock hides behind a normal blood pressure.** Respiration and pulse speed up, the skin becomes pale, cool and clammy, capillary refill slows, nausea and anxiety appear. A reversible phase, revealed only by serial vital signs.
3. **Progressive shock, then irreversible.** When blood pressure drops, a vicious circle sets in and the shock becomes self-sustaining. At this stage, the only response is the fastest possible evacuation. A child compensates for a long time (up to 40% of blood volume lost) and then collapses suddenly.
4. **Management starts with the cause.** Control the hemorrhage, open the airway, treat the anaphylaxis. Then: lying position (Fowler's if breathing is difficult, recovery position if unconscious and breathing), warmth, calm, splinting of fractures, constant monitoring of vital signs.
5. **Nothing by mouth at first, except during a prolonged evacuation.** The patient may vomit. If the evacuation drags on and the patient is conscious and alert, offer small amounts of water and food. The acute stress reaction mimics the early signs of shock, in the patient as well as in the first aider.

LEARNER'S MANUAL	FIELD MANUAL	IN CLASS
Section 5, Shock · Online encyclopedia	**Pages 30 to 33, shock and positioning**	**Serial vital signs, body positions, hypothermia wrap**

04 Moving patients

Some moves are urgent, others wait for the evacuation

Two moments for moving a patient. Very early, if an imminent danger threatens the patient or the rescuers. Much later, once the patient has been assessed, treated and protected, to evacuate.

1. **Imminent danger justifies moving before assessing.** Avalanche, rising water, fire, rockfall, gas, unstable vehicle. Only in these cases does the move come before the primary survey. Otherwise, a trauma patient is not moved until the spine has been considered.
2. **The BEAM: lift and move as a unit.** The patient is moved in the position found, head and torso aligned, by a coordinated team. A single leader gives the instructions and the signal. Practised in class.
3. **Choose the technique according to three criteria.** The patient's condition (can they walk, help, tolerate a position), the number and build of the carriers, the terrain and the distance. One-rescuer carry, two-rescuer carry, backpack carry, rope coil or improvised chair for short distances.
4. **An improvised litter takes a long time to build.** Tarp, jackets, framed backpacks, poles. Practise at least one method suited to your equipment and your environment ahead of time, rather than improvising on the spot.
5. **Transport is a phase of care, not a break.** Count on half a kilometre per hour or less by litter. Plan carrier rotations, protect the patient from the weather and keep access to the injuries and vital signs so you can reassess along the way.

LEARNER'S MANUAL
Section 15, Transporting and Moving Patients · Online encyclopedia

FIELD MANUAL
Pages 49 to 56, log rolls, BEAM, litters

IN CLASS
BEAM, tarp litter, backpack-and-pole chair; the other techniques are covered by video awareness

05 Evacuation considerations

Deciding when, how, and with which resources

> **The decision to evacuate flows from the PAS form: the problem list tells you whether the evacuation is urgent, semi-urgent, or whether the patient can stay with the group.** The rest is a matter of logistics and safety.

1. **Eight elements to review, every time.** The condition of the patients and the group, the location, physical resources, the type of assistance required, the type of assistance available, the weather, the time of day, then messengers and written messages.
2. **The concrete questions about the patient.** Can they walk? Are spinal precautions needed? A splinted limb? Which destination, and how long to get there? Can they wait until morning?
3. **The group is part of the equation.** Who carries, who stays, who is responsible for those who stay if the leader leaves? An exhausted, wet or frightened group can produce a second patient. Waiting for optimal conditions is sometimes the best decision.
4. **Messengers, never alone.** At least two, ideally four, each equipped to spend a night out and each carrying a copy of the same written message.
5. **The written message says everything you will not be able to say again.** One PAS form per patient with the care already given, exact date, time and location, equipment and personnel needs, names of those who stayed behind, supplies and how long they will last, your plans, the meeting point, a landing zone or motorized access point, and any other evacuation option.

LEARNER'S MANUAL
Section 6.6, Evacuation Considerations · Online encyclopedia

FIELD MANUAL
Pages 46 and 47, when to evacuate and landing zone

IN CLASS
Evacuation planning from the PAS form, scenarios

ASSESSMENT

06 Communication

A method adapted to wilderness first aid

> **Communicating means conveying the essentials, without ambiguity, to someone who is not on scene, often through a limited channel (satellite, radio, messenger), to get the right help, to the right place, within the right time frame.**

1. **A structured message, not a story.** Existing hospital models assume a medical contact, a stable line and an already-assessed patient. In the backcountry, the CLEAR method applies the same principles, adapted to your context and your PAS form.
2. **Prepare the message before calling.** PAS form in hand, precise position (coordinates, landmarks, access), the patient's condition in one sentence, what has been done, what you are asking for. A satellite call often drops after a few seconds: say the essentials first.
3. **Know your means and their limits.** Cell phone, satellite phone, satellite messenger, radio, distress beacon, runners. Each has a delay, a range and a failure mode. The emergency plan says which one to use and which number to call.
4. **Confirm that the message was understood.** Have the critical elements repeated back: location, number of patients, severity, evacuation method requested, next contact. Agree on the time of the next call and on what happens if you do not call back.
5. **Document what you observe and what you do.** Time of each call, contact person, content, decisions. This log complements the PAS form and becomes the thread of the handover to rescue services.

LEARNER'S MANUAL	FIELD MANUAL	NOTE
N/A	N/A	CLEAR: Context, Location, Evaluation, Actions, Request